



The Society of Addiction Medicine Physician Associates Response to the HHS Request for Information on the Chronic Disease of Addiction

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The Society of Addiction Medicine Physician Associates (SAMPA) appreciates the opportunity to respond to the U.S. Department of Health and Human Services (HHS) Request for Information (RFI) on the chronic disease of addiction and the Great American Recovery Initiative. SAMPA represents physician associates/assistants (PAs) who provide evidence-based prevention, treatment, and recovery services for individuals with substance use disorders (SUD) in every state and every practice setting, including the rural and underserved communities this RFI prioritizes.

SAMPA's central message is a workforce message. There were 158,470 board-certified PAs in the United States at the end of 2021, with more than 111,000 in active clinical practice ([Hooker et al., Human Resources for Health, 2024](#)). Congress removed the principal federal barrier to PA prescribing of buprenorphine when Section 1262 of the Consolidated Appropriations Act, 2023 eliminated the DATA 2000 waiver, and SAMHSA's 2024 revision of 42 CFR Part 8 opened opioid treatment program (OTP) practice to appropriately licensed PAs. Yet patient-level access has not grown in proportion to the expanded prescriber pool, because a patchwork of state scope-of-practice restrictions, payment differentials, and unfinished federal telehealth rulemaking continues to sideline this workforce. Each recommendation below identifies an existing-authority lever HHS can use to close that gap and deliver on Executive Order 14379.

Question 1. Programs or interventions with rigorous, empirical evidence of effectiveness

1. Title: Expand Medication for Opioid Use Disorder (MOUD) Accessibility.

i. **Type of Activity:** Substance use treatment and recovery.

ii. **Date:** 2015–present.

iii. **Link:** [Sordo et al., BMJ 2017](#); [Volkow et al. 2015](#); [Wakeman et al., JAMA Netw Open 2020](#); [JAMA Psychiatry 2022](#)

iv. **Description:** Medications for opioid use disorder (MOUD) — methadone, buprenorphine, and naltrexone — are associated with substantial reductions in all-cause and overdose mortality and are the gold standard of care for opioid use disorder (OUD). Low-barrier delivery models, including same-day initiation and telehealth, improve treatment engagement and retention and remove structural obstacles faced by rural and underserved populations. The evidence for MOUD is among the strongest in all of addiction medicine and supports scaling accessible MOUD as the core of the Great American Recovery. Federal policy has already moved in this direction; the remaining task, addressed in our answers to Questions 2 and 4, is ensuring the full licensed workforce can deliver it.

v. **Statutory authority:** [21 U.S.C. § 823\(g\)](#); [SUPPORT for Patients and Communities Act \(P.L. 115-271\)](#); [Section 1262 of the Consolidated Appropriations Act, 2023 \(P.L. 117-328\)](#); [42 U.S.C. § 290dd-2](#); [42 CFR Part 8](#); [21 CFR Part 1306](#).

2. Title: PA- and NP-Driven Expansion of Rural Buprenorphine Access Following CARA 2016.

i. **Type of Activity:** Substance use treatment and recovery; workforce policy with demonstrated outcomes.

ii. **Date:** 2017–present.

iii. **Link:** [Barnett, Lee & Frank, Health Affairs 2019](#); [MACPAC 2019](#)

iv. **Description:** The Comprehensive Addiction and Recovery Act of 2016 (CARA) first authorized PAs and nurse practitioners (NPs) to obtain federal waivers to prescribe buprenorphine, and the results constitute rigorous evidence that expanding the eligible workforce expands access. From 2016 to 2019, buprenorphine-waivered clinicians per 100,000 population in rural areas increased 111 percent; PAs and NPs accounted for more than half of that increase and were the first waived clinicians in 165 rural counties that are home to 5.7 million Americans. A MACPAC-commissioned analysis found buprenorphine prescriptions to Medicaid beneficiaries grew 12 percent in the year after implementation, twice the rate of growth for all patients. This natural experiment demonstrates that practitioner-inclusive federal policy is itself an evidence-based access intervention, particularly for the rural communities served by HRSA's Rural Communities Opioid Response Program.

v. **Statutory authority:** [Comprehensive Addiction and Recovery Act of 2016 \(P.L. 114-198, § 303\)](#); [SUPPORT Act \(P.L. 115-271\)](#); [Section 1262 of the Consolidated Appropriations Act, 2023 \(P.L. 117-328\)](#).

3. Title: Expand Psychiatric Collaborative Care Models (CoCM).

i. **Type of Activity:** Care for co-occurring mental illness and chronic disease of addiction.

ii. **Date:** 2002–present.

iii. **Link:** [Archer et al. / AIMS Center evidence base; JAMA Internal Medicine 2017](#)

iv. **Description:** The Psychiatric Collaborative Care Model (CoCM) integrates behavioral health services into primary care through patient-centered team care, validated in more than 90 randomized controlled trials. It is directly relevant to individuals with SUD, who frequently have co-occurring mental health conditions that complicate treatment and recovery. CoCM improves access to psychiatric expertise, supports longitudinal management of addiction as a chronic disease, and increases treatment engagement among patients with co-occurring conditions. PAs, who deliver a large share of primary care in rural areas, are natural CoCM team clinicians when payment rules allow them to participate fully.

v. **Statutory authority:** [Social Security Act § 1833](#), [§ 1834\(m\)](#), [§ 1842\(b\)\(18\)](#); [42 U.S.C. § 1395m](#) (Medicare payment for behavioral health integration services).

Question 2. Policies or changes to federal programs, using existing funding, that might improve outcomes

1. Title: Make Telehealth Flexibilities for SUD Treatment Permanent — with Practitioner-Neutral Language — Before the December 31, 2026 Telemedicine Cliff.

i. Type of Activity: Substance use treatment and recovery; regulatory action using existing authority.

ii. Date: 2020–present.

iii. Link: [90 FR 6504 \(buprenorphine telemedicine final rule\)](#); [Fourth Temporary Extension through Dec. 31, 2026](#); [DEA Special Registration NPRM announcement](#)

iv. Description: The DEA/HHS final rule Expansion of Buprenorphine Treatment via Telemedicine Encounter (90 FR 6504), effective December 31, 2025, permanently allows DEA-registered practitioners — in practitioner-neutral language that includes PAs — to initiate an initial six-month supply of buprenorphine via telemedicine, including audio-only encounters, subject to PDMP review. Broader controlled-substance telemedicine prescribing, however, rests on a fourth temporary extension that expires December 31, 2026, and the proposed Special Registrations for Telemedicine framework (90 FR 6541) remains unfinalized. SAMPA urges HHS to work with DEA to finalize the special registration rule before the deadline, preserving the proposal's inclusion of mid-level practitioners for Schedule III–V prescribing and reconsidering physician-only limitations where no clinical evidence supports excluding PAs practicing within state law. Telehealth is disproportionately how behavioral health PAs deliver care — 62.7 percent of PAs in psychiatry provide telemedicine services, nearly twice the rate of the profession overall — so an abrupt telemedicine cliff would fall hardest on the patients of exactly the workforce HHS needs to retain.

v. Statutory authority: [21 U.S.C. § 802\(54\)\(E\)](#) (special registration for telemedicine); [21 U.S.C. § 829\(e\)](#) (Ryan Haight Act); [21 CFR § 1306.51](#); [42 CFR Part 12](#); [SUPPORT Act § 3232](#).

2. Title: Launch a Federal–State MOUD Alignment Initiative to Remove Stranded and Duplicative State Barriers to Prescribing.

i. Type of Activity: Substance use treatment and recovery; guidance, technical assistance, and grant best practices using existing funding.

ii. Date: 2023–present (post–MAT Act).

iii. Link: [SAMHSA, Waiver Elimination \(MAT Act\)](#); [Andraka-Christou et al., J Subst Abuse Treat 2022](#); [Tenn. Code § 53-11-311 \(2024\)](#)

iv. Description: Section 1262 removed the federal waiver, but SAMHSA itself cautions that prescribing is permitted only “if permitted by applicable state law,” and state law has not kept pace. Tennessee statute still makes physicians the only default buprenorphine prescribers and conditions PA prescribing on an “active DATA 2000 waiver” — a federal credential Congress abolished in 2023 — alongside 100-percent physician chart review, supervision ratio caps, 16 mg dosing ceilings, and patient caps that physicians do not face; Kentucky ties PA buprenorphine prescribing to delegation by a supervising physician who personally prescribes buprenorphine; and five states (Louisiana, Maine, Ohio, Oregon, Virginia) conditioned PA prescribing on the supervising physician holding a now-nonexistent X-waiver. These policies have measurable consequences: the PA/NP share of buprenorphine treatment episodes ranges from 0.4 percent in Alabama to 57.2 percent in Montana, and full scope of practice is significantly associated with greater MOUD prescribing, with effects concentrated in rural counties. Without new funding, HHS can (a) issue SAMHSA/HRSA Dear Colleague guidance and State Opioid Response and block grant best practices encouraging states to repeal MOUD-specific

advanced practice restrictions and stranded DATA-waiver references, (b) direct ASPE to publish a federal–state alignment report inventorying state laws that conflict with Section 1262 and 42 CFR Part 8, and (c) make state alignment a scored factor in discretionary grant competitions, as SAMHSA already does for the 100 hardest-hit counties.

v. Statutory authority: [Section 1262, P.L. 117-328](#); [42 U.S.C. § 290aa](#) (SAMHSA authorities); [21st Century Cures Act § 1003](#) (State Opioid Response); [42 U.S.C. §§ 300x-21 et seq.](#) (SUBG); [42 U.S.C. § 254c](#) (RCORP); [P.L. 119-21](#) (Rural Health Transformation Program).

3. Title: Fully Implement the 2024 OTP Rule's Practitioner Provisions and Encourage State Opioid Treatment Program Alignment.

i. Type of Activity: Substance use treatment and recovery; regulatory implementation and state technical assistance.

ii. Date: 2024–present.

iii. Link: [89 FR 7528 \(42 CFR Part 8 final rule\)](#); [Health Affairs 2025 \(state OTP restrictiveness\)](#); [Pew 2022 \(state OTP regulations\)](#)

iv. Description: SAMHSA's 2024 revision of 42 CFR Part 8 redefined “practitioner” so that appropriately licensed PAs may order methadone within OTPs, and the rule's preamble candidly acknowledges that some states still bar PAs from ordering methadone without physician supervision notwithstanding the federal flexibility. State-layered OTP regulation measurably constrains access: as of 2018, 80 percent of U.S. counties had no OTP; nineteen states plus D.C. impose certificate-of-need barriers on new OTPs and West Virginia maintains a statutory moratorium; and 2025 peer-reviewed evidence shows ZIP codes in low-restrictiveness states have more than twice the OTP density and methadone shipments of comparable ZIP codes across the border. SAMPA recommends that SAMHSA issue implementation guidance and provide technical assistance to State Opioid Treatment Authorities specifically encouraging adoption of the federal practitioner definition, and that HHS support future consideration of practitioner-neutral eligibility — consistent with state supervision law — in any methadone modernization framework, in contrast to proposals such as the Modernizing Opioid Treatment Access Act (S. 644) that limit new prescribing pathways to board-certified physicians alone. Fully staffing OTPs with the licensed workforce Congress and SAMHSA have already authorized is the fastest existing-funding route to methadone access in the 100 hardest-hit counties.

v. Statutory authority: [21 U.S.C. § 823\(h\)](#); [42 CFR Part 8 \(89 FR 7528\)](#); [Section 1252, P.L. 117-328](#); [42 U.S.C. § 290bb-2a](#).

Question 3. Mitigating stigma against Americans seeking addiction treatment and recovery

1. Title: Leverage the MATE Act and Health Professions Education to Reduce Clinical Stigma in SUD Care.

i. Type of Activity: Education and training; stigma reduction across the health professions.

ii. Date: 2023–present.

iii. **Link:** [DEA MATE Act training letter; Moreland et al., J Am Assoc Nurse Pract 2025](#)

iv. **Description:** Stigma within health care itself remains a leading reason patients do not initiate or continue addiction treatment, and clinician education is among the best-evidenced remedies. The MATE Act (Section 1263) already requires a one-time eight hours of SUD training for every DEA registrant, and DEA recognizes the American Academy of Physician Associates among the accredited providers of that training. HHS can amplify this existing infrastructure at no new cost by partnering with certifying and membership organizations — including PA organizations such as SAMPA — to embed person-first, recovery-oriented, evidence-based content in MATE-qualifying curricula and in health professions programs funded under Title VII of the Public Health Service Act. Recent curricular research demonstrates the payoff: when 120 PA and NP students at three universities completed the eight-hour MOUD training within their degree programs, knowledge, favorable attitudes toward MOUD, perceived behavioral control, and intention to prescribe buprenorphine, methadone, and naltrexone all significantly increased (Moreland et al., 2025). SAMHSA's own 2024 Part 8 rule, which removed stigmatizing and outdated language from federal regulation, provides the model and precedent for a department-wide language modernization across grants, guidance, and program materials.

v. **Statutory authority:** [Section 1263, P.L. 117-328](#) (MATE Act); [Public Health Service Act Title VII \(42 U.S.C. § 292 et seq.\)](#); [42 CFR Part 8](#) (89 FR 7528).

2. Title: Expand Peer Recovery Support Services to Normalize and Celebrate Recovery.

i. **Type of Activity:** Recovery support; stigma reduction.

ii. **Date:** 2018–present.

iii. **Link:** [SAMHSA recovery support services resources](#)

iv. **Description:** Peer recovery support specialists — people in sustained recovery serving others — reduce stigma by making recovery visible, credible, and celebrated, directly advancing E.O. 14379's call to foster a culture that celebrates recovery. Peer services are associated with improved engagement and retention in SUD treatment and are already fundable through the State Opioid Response grants, the Substance Use Block Grant, and the recovery housing investments highlighted in this RFI. SAMPA recommends HHS strengthen peer integration requirements and technical assistance across these existing programs and encourage clinical settings — including PA-led rural practices — to pair MOUD treatment with peer support so that medication and recovery culture reinforce one another. Embedding peers alongside prescribing clinicians signals to patients that treatment-seeking is an act the community honors rather than hides.

v. **Statutory authority:** [21st Century Cures Act § 1003](#); [42 U.S.C. §§ 300x-21 et seq.](#); [42 U.S.C. § 290bb-25](#).

Question 4. Addressing the practitioner supply problem so every American can find affordable, covered treatment

The RFI correctly identifies shortfalls of roughly 77,050 addiction counselors and 99,780 mental health counselors, and the psychiatrist deficit is projected at 10,500–28,500 by 2030. SAMPA respectfully urges HHS to treat PAs as a central part of the answer: a large, certified,

prescribing workforce that already grew its psychiatry footprint from 630 PAs in 2013 to 2,262 in 2021 and that demonstrably brought MOUD to rural counties no other prescribers served.

1. Title: Recognize and Recruit PAs Across HHS Behavioral Health Workforce Programs.

i. Type of Activity: Workforce development using existing programs.

ii. Date: 2018–present.

iii. Link: [NHSC SUD Workforce Loan Repayment Program](#); [Hooker et al. 2024 \(PA psychiatry workforce\)](#)

iv. Description: PAs are already an eligible discipline for the National Health Service Corps SUD Workforce Loan Repayment Program (up to \$75,000 for three years of full-time service at approved SUD sites in shortage areas), but PA participation in addiction settings remains far below its potential. Using existing appropriations, HRSA can market these awards directly to the PA pipeline, prioritize addiction-treatment sites that employ PAs, and ensure PAs are explicitly counted in HRSA behavioral health workforce projections so the Department's own supply models reflect the full prescriber workforce. HHS should also support emerging PA credentialing in addiction medicine — analogous to the existing Certificate of Added Qualifications framework in psychiatry — so that competence in this field is visible to employers, payers, and patients. Every federal workforce document that says “physicians and nurses” and omits PAs quietly understates national treatment capacity.

v. Statutory authority: [Public Health Service Act §§ 331–338 \(42 U.S.C. § 254d et seq.\)](#); [SUPPORT Act § 7071](#) (Loan Repayment Program for SUD Treatment Workforce); [PHSA Title VII](#).

2. Title: Use HHS Guidance and Grantmaking to Encourage States to Remove Duplicative, MOUD-Specific Restrictions on PA Practice.

i. Type of Activity: Workforce policy; federal–state alignment using existing authority.

ii. Date: 2023–present.

iii. Link: [Spetz et al., JAMA 2019](#); [Harrison et al., Med Care Res Rev 2022](#); [Saloner et al. \(post-X-waiver barriers\)](#); [Andrilla et al., J Rural Health 2020](#)

iv. Description: After the X-waiver's elimination, the number of clinicians dispensing buprenorphine rose immediately and kept growing across every specialty category, yet patient-level gains were limited — evidence that the binding constraints are now downstream of federal prescribing law. Peer-reviewed analyses identify state supervision and collaboration requirements specific to buprenorphine as measurable suppressors of PA prescribing, and no state yet permits PAs to prescribe this Schedule III medication on the same terms as physicians. HHS cannot preempt state scope-of-practice law, but it can do what it already does elsewhere: publish best-practice guidance, condition technical assistance, and use grant scoring preferences to encourage states to align supervision requirements for MOUD with those for other Schedule III medications, remove obsolete waiver-linked provisions, and count PAs in Medicaid managed care network adequacy standards for SUD services. In a national survey of buprenorphine-waivered NPs and PAs, the most commonly reported barriers to incorporating treatment were concerns about diversion or misuse (50.9 percent), lack of available mental health or psychosocial support services (44.1 percent), and

insurance issues such as prior authorization (41.8 percent) — with rural clinicians significantly more likely to report lacking specialty backup and mental health support — barriers that HHS technical assistance and existing grant programs can directly address (Andrilla et al., J Rural Health 2020). The Department's Rural Health Transformation Program and RCORP investments will underperform in the 2,000 rural counties they cover if the largest available prescriber workforce remains legally sidelined in the very states with the fewest physicians.

v. Statutory authority: [42 U.S.C. § 290aa](#); [21st Century Cures Act § 1003](#); [42 U.S.C. § 254c](#); [Social Security Act § 1902](#), [§ 1932](#) (Medicaid state plan and managed care standards); [P.L. 119-21](#).

3. Title: Align Payment Policy So PA-Delivered Addiction Care Is Financially Sustainable.

i. Type of Activity: Payment policy under existing Medicare/Medicaid authority.

ii. Date: Ongoing.

iii. Link: [Harrison et al. 2022 \(Medicaid pay parity association\)](#)

iv. Description: Medicare reimburses services personally performed by PAs at 85 percent of the physician fee schedule, and Medicaid payment policies for PA-delivered MOUD vary widely by state; peer-reviewed evidence associates Medicaid payment parity with significantly greater PA/NP buprenorphine treatment, with effects concentrated in rural counties. Within existing authority, CMS can issue State Medicaid Director guidance highlighting payment parity for MOUD services as a best practice, confirm that PAs can fully participate in and bill behavioral health integration and Collaborative Care Model codes, and examine whether payment differentials undermine the sustainability of addiction practices in shortage areas. Low-margin rural treatment settings are precisely where an unwarranted 15 percent payment differential determines whether a PA-staffed access point exists at all. Payment alignment is a workforce policy: clinicians go where their services are sustainably reimbursed.

v. Statutory authority: [Social Security Act § 1842\(b\)\(18\)](#), [§ 1861\(s\)\(2\)\(K\)\(i\)](#), [§ 1833](#), [§ 1902](#); [42 U.S.C. § 1396a](#).

Question 5. Strengthening evaluation, data modernization, and AI-enabled performance measurement

1. Title: Standardize Outcome Measures — Including Prescriber-Type Data — Across Federally Funded SUD Programs.

i. Type of Activity: Data modernization and program evaluation.

ii. Date: 2025–present.

iii. Link: [ASTP/ONC–SAMHSA Behavioral Health IT initiative](#)

iv. Description: HHS's \$20 million Behavioral Health IT initiative and the standardized behavioral health data elements it is piloting give the Department an existing vehicle to require consistent outcome measures — treatment initiation, MOUD retention, overdose, mortality — across SAMHSA, HRSA, and CMS-funded programs. SAMPA recommends the standardized data set include practitioner type and practice setting so the Department can measure, rather than assume, the contribution of each discipline to

access and outcomes; today's federal datasets frequently obscure PA-delivered care through incident-to billing and physician-attributed claims. Uniform measures would let HHS compare grant portfolios in near real time and would generate the evidence base states need when they consider aligning their practice laws with federal policy. Measurement that is workforce-blind cannot inform workforce policy.

v. Statutory authority: [Foundations for Evidence-Based Policymaking Act of 2018 \(P.L. 115-435\)](#); [PHSA § 505 \(42 U.S.C. § 290aa-4\)](#); [HITECH Act \(P.L. 111-5\)](#); [42 U.S.C. § 300jj et seq.](#)

2. Title: Build Near-Real-Time MOUD Access Dashboards and Apply AI to Continuous Policy Evaluation.

i. Type of Activity: Advanced analytics and AI-enabled performance measurement.

ii. Date: 2026 forward.

iii. Link: [Am J Prev Med 2025 \(interrupted time-series of X-waiver elimination\)](#)

iv. Description: Researchers using national pharmacy claims demonstrated within months that X-waiver elimination grew the prescriber pool by roughly 1,626 clinicians immediately — while patient-level gains lagged — the kind of insight HHS should generate internally and continuously rather than await in journals. Using existing data authorities, HHS can combine DEA registration data, PDMP interfaces, Medicare/Medicaid claims, and the BHIT data elements into near-real-time dashboards of MOUD prescribers and treated patients by county and clinician type, with AI-assisted anomaly detection flagging counties where prescriber growth is not translating into patient access. Such a system would let the Department evaluate every policy change — telehealth rules, Part 8 implementation, grant investments — on a continuing basis and target technical assistance to the 100 hardest-hit counties the Administration has prioritized. Evaluation at this cadence turns the Great American Recovery from a grant portfolio into a learning system.

v. Statutory authority: [Foundations for Evidence-Based Policymaking Act \(P.L. 115-435\)](#); [42 U.S.C. § 290aa\(d\)](#); [Executive Order 14379](#); [SUPPORT Act \(P.L. 115-271\)](#) data provisions.

Conclusion

SAMPA appreciates the opportunity to respond to this RFI. The nation has already built much of the legal architecture for accessible, evidence-based addiction treatment: Congress eliminated the X-waiver, SAMHSA modernized 42 CFR Part 8, and HHS and DEA have opened a permanent telemedicine pathway for buprenorphine. What remains is alignment — finalizing the pending telemedicine framework with practitioner-neutral language, encouraging states to retire obsolete and duplicative restrictions on PA practice, aligning payment with team-based care, and measuring outcomes by clinician type so policy can follow evidence. PAs stand ready to serve as a principal workforce of the Great American Recovery, particularly in the rural and hard-hit communities where they are so often the first — and sometimes the only — addiction treatment clinicians available. We welcome continued collaboration with HHS and its operating divisions on each recommendation above.

Respectfully submitted,

The Society of Addiction Medicine Physician Associates (SAMPA)